Session 3 – Elicitation to Requirements Candidates

CityCare Clinic Scheduling Upgrade

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Lab 1 – Stakeholder Analysis and Elicitation Plan

1. Stakeholder List

- Clinic Director (Sponsor)
- Reception / Scheduling Staff
- Doctors
- Nurses
- Patients (New & Returning)
- Call Center Staff
- Billing / Insurance Team
- IT Operations
- Security Team
- Compliance / Legal
- EHR Vendor
- Facilities Manager
- Training Team

2. Power–Interest Categorization

1. Manage Closely

- Clinic Director
- Reception Staff
- Doctors
- IT Ops
- Compliance

2. Keep Satisfied

- Billing Team
- Security Team

3. Keep Informed

- Nurses
- Patients

4. Monitor

- Facilities
- Training Team

3. Elicitation Plan

Objective: Understand scheduling pain points and reminder requirements to reduce no-shows and double-bookings.

5. Stakeholders Engaged:

- Reception Staff
- Doctors
- IT Ops
- Compliance
- Patients

6. Techniques Used:

- Interviews
- Shadowing
- Surveys
- Document Review

Key Elicitation Questions

7. Walk me through scheduling a new patient.
8. What happens when two staff edit the sheet at the same time?
9. Tell me about the last double-booking incident.
10. Which visit types require buffer time?
11. How are reminders currently sent?
12. What happens when patients don't answer reminder calls?
13. How do patients select reminder preferences?
14. What privacy rules apply to SMS?
15. What monitoring is required for patient-facing systems?
16. What reports are needed weekly?

Lab 2 – Transcript to Requirements Candidates

1. Summary of Key Needs and Pain Points

- Scheduling is done using a shared spreadsheet.
- EHR updates are delayed and sometimes forgotten.
- Double-bookings occur due to simultaneous editing.
- Buffer time between visit types is not enforced.
- Reminder calls are manual and inconsistent.
- Patients prefer different reminder channels.
- Patient-facing systems require SSO authentication.
- SMS messages cannot contain medical details.

2. Requirements Candidates

ID	Statement	Type	Rationale	Source Quote	Acceptance	Open Questions
RC-001	Provide centralized appointment scheduling replacing shared spreadsheet.	Functional	Prevent double-bookings.	"Double-bookings happen when two staff edit the sheet at the same time."	System prevents duplicate bookings during simultaneous edits.	How will concurrent editing be handled?
RC-002	Automatically update EHR during appointment booking.	Functional	Avoid forgotten manual updates.	"We put a note in the EHR later. Sometimes we forget."	Appointment appears in EHR immediately after confirmation.	Is integration API available?
RC-003	Enforce minimum 15-minute buffer between certain visit types.	Functional	Doctor scheduling constraint.	"I need at least 15 minutes between certain visit types."	System blocks bookings violating buffer rule.	Which visit types require buffer enforcement?
RC-	Send automated	Functional	Reduce missed	"Patients often miss	Reminder is	What should be

004	appointment reminders.		appointments.	appointments. We call them the day before."	automatically triggered before appointment time.	reminder timing?
RC-005	Allow patients to select SMS or Email reminder preference.	Functional	Respect communication preferences.	"Some patients prefer text reminders; others only want email."	Preference stored and used during reminder dispatch.	Can patients update preference anytime?
RC-006	SMS reminders must not include medical details.	Constraint	Compliance requirement.	"Do not include medical details in SMS; keep messages generic."	SMS contains only generic information.	Are there restrictions for email?
RC-007	All patient-facing features shall use SSO authentication.	Non-Functional (Security)	IT security policy.	"Anything patient-facing must use our single sign-on and be monitored."	Users authenticate only via approved SSO provider.	Which SSO provider is implemented?
RC-008	Maintain audit logs for appointment-related actions.	Non-Functional (Monitoring/Compliance)	Monitoring and compliance requirement.	"We also need audit logs."	Logs capture user ID, timestamp, and action performed.	What is the audit log retention period?

Follow-Up Questions

17. Which visit types require buffer enforcement?

18. What is the exact reminder timing (24h, 2h, etc.)?
19. What is the audit log retention period?

Lab 3 – Problem Statement and Scope

Problem Statement

The current situation is that CityCare Clinic schedules appointments using phone calls and a shared spreadsheet, with manual reminder processes. This causes double-bookings, missed appointments, delayed EHR updates, and reduced visibility for clinicians. The goal is to implement an improved scheduling and reminder system that reduces errors, decreases no-shows, and improves operational efficiency and patient experience.

Success Measures

- Reduction in double-booking incidents
- Reduction in no-show rate
- Faster time-to-book appointments
- Reduced manual workload for staff

Constraints

- No medical details in SMS messages
- Mandatory single sign-on (SSO) authentication
- Audit logging and monitoring requirements

Scope Definition

In Scope

- Centralized appointment scheduling system
- Automated appointment reminders
- Reminder preference selection
- Buffer rule enforcement
- EHR integration
- Audit logging and monitoring

Out of Scope

- Full EHR system replacement
- Clinical diagnosis functionality
- Billing system redesign
- Marketing campaigns

Assumptions

- EHR integration API is available
- Patients provide valid contact details
- Staff will receive training on new system

Dependencies

- IT infrastructure readiness
- SSO provider configuration

- Compliance approval for messaging content